



Doron's Health Report

Assessment: Jun 29, 2026 · Generated: Jun 29, 2026

PATIENT

Doron Patient3
Male · 49 years

PREPARED BY

Dr. Doron Owner1
At Doron's #2

Your Health Status

"Since your last assessment, the core metabolic and kidney themes are still present, but the newer chart information adds important context. Your strongest objective signals remain reassuring blood counts, normal thyroid screening, and normal liver chemistries on the last available labs. The main issue is still an insulin-resistance pattern with elevated fasting insulin, high triglycerides, low HDL, and borderline LDL on the last fasting panel, and the older 2023 results show this has been a longer-running pattern. Kidney function still looks mildly reduced rather than clearly normal, and that matters more now that hypertension is documented. The hormone picture is less mysterious than before because the testosterone rise is now more consistent with active enclomiphene treatment, but it still needs monitoring. New charted diagnoses of hypertension, psoriasis, ADHD, and depression also widen the prevention focus beyond labs alone. The good news is that this still looks like a window for active risk reduction rather than advanced organ failure, but the next step is to refresh the stale lab picture and fill the biggest gaps, especially actual blood pressure readings, ApoB, Lp(a), urine albumin, cystatin C, and a formal sleep assessment."

— Dr. Doron Owner1

Your Story

Patient-reported goals and concerns driving this assessment.

- **Kidney monitoring:** You want to keep a close eye on your kidney function and understand whether the prior creatinine and eGFR pattern represents a stable mild issue or something that needs stronger protection.
- **Glycemic control:** You want a clearer read on your blood sugar control and insulin resistance, not just whether you meet a diabetes cutoff.
- **Lipid management:** You want to improve your cholesterol profile and better define your long-term cardiovascular prevention strategy.
- **Hormone evaluation:** You want to understand your reproductive hormone pattern better, especially given the short-term variability in testosterone, LH, FSH, and estradiol.
- **Blood pressure control:** You want to know whether your blood pressure is truly controlled and how it fits into your broader prevention plan.
- **Mental health:** You want your depression and ADHD care to be part of the overall longevity plan, not treated as separate from the rest of your health.

Your Goals

1 Lower insulin and triglycerides

To Chronic insulin resistance

Metabolic Health, Liver Health, Nutrition & Vitamins · 24 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
Hemoglobin A1c	5.5 %	5.2 %	24 weeks
Insulin	27.8 uIU/mL	18 uIU/mL	12 weeks
Triglycerides	187 mg/dL	140 mg/dL	12 weeks

2 Lower lipid and BP risk

To Hypertension plus dyslipidemia

Cardiovascular Health, Brain Health, Inflammation · 24 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
Non-HDL Cholesterol	129 mg/dL	100 mg/dL	12 weeks
HDL Cholesterol	39 mg/dL	45 mg/dL	24 weeks
LDL-Cholesterol	101 mg/dL	80 mg/dL	12 weeks
Home blood pressure log	not documented	completed	3 weeks

3 Protect kidney filtration

To Borderline renal function

Kidney Health, Cardiovascular Health · 24 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
Creatinine	1.33 mg/dL	1.20 mg/dL	24 weeks
eGFR	66 mL/min/1.73m2	75 mL/min/1.73m2	24 weeks

4 Improve sleep and brain recovery

To Sleep and psychiatric strain

Sleep Health, Brain Health, Emotional & Social Health · 12 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
THC use	nightly	reduced or clinician-reviewed	12 weeks
Psychiatric medication review	not recently reconciled	completed	6 weeks
Sleep assessment	not completed	completed	4 weeks

5 Lower fatty-liver risk

To Prior ALT elevation

Liver Health, Metabolic Health, Nutrition & Vitamins · 24 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
Insulin	27.8 uIU/mL	15 uIU/mL	24 weeks
Triglycerides	187 mg/dL	120 mg/dL	24 weeks

6 Protect knees and reserve

To Active knee OA

Musculoskeletal Health, Nutrition & Vitamins, Blood Health · 12 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
Strength training frequency	3 sessions/week	4 sessions/week	12 weeks
DXA scan	untested	completed	12 weeks

7 Monitor hormone therapy response

To Active hormone therapy follow-up

Thyroid & Hormone Health · 12 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
Estradiol	53 pg/mL	35 pg/mL	12 weeks

8 Address psoriasis and prevention gaps

To Psoriasis and screening refinement

Skin Health, Inflammation, Gut Health, Cancer Screening · 12 weeks

WHAT WE'RE MEASURING	WHERE YOU ARE NOW	WHERE WE'RE HEADING	TIMEFRAME
Full-body skin exam	not documented	completed	12 weeks
Hereditary cancer risk review	not completed	completed	12 weeks

Your Plan

Based on your results and goals, your plan includes 1 nutrition, 5 lifestyle, and 1 medications. Everything here is designed to work together to help you feel better and stay on track. Your care team will walk you through each step.

Nutrition 1
Build protein-fiber meals
Lifestyle 5
Add cardio and lifting
Track your home BP
Set a fixed wake time
Reduce nightly THC
Start knee-smart training
Medications 1
Continue enclomiphene as prescribed

ORDERS

Labs

Fasting metabolic follow-up panel

Lipid risk refinement panel (fasting lipid panel + ApoB + Lp(a))

Kidney clarification panel

Liver-metabolic reassessment panel (CMP + GGT + CBC + fasting insulin + HbA1c + lipid panel)

Male hormone surveillance panel (estradiol + total testosterone + free testosterone + SHBG + CBC/hematocrit + CMP)

Imaging

Liver ultrasound with elastography

DEXA body composition and bone density scan

Referrals

Ambulatory blood pressure monitoring or clinician-validated hypertension follow-up

Sleep medicine evaluation

Psychiatry or prescribing-clinician medication review

Physical therapy referral for bilateral knee osteoarthritis

Dermatology referral for psoriasis staging and full-body skin exam

Cancer genetics referral for hereditary risk review

Timeline & Follow-up

Here's what happens next and when to expect check-ins.

Now

- **Build protein-fiber meals:** Fiber ramp
- **Build protein-fiber meals:** Meal structure
- **Build protein-fiber meals:** Meal structure
- **Add cardio and lifting:** Base volume
- **Add cardio and lifting:** Strength habit
- **Add cardio and lifting:** Build walking base
- **Add cardio and lifting:** Aerobic base
- **Track your home BP:** Run monitor
- **Track your home BP:** Collect series
- **Track your home BP:** Set technique
- **Set a fixed wake time:** Anchor mornings
- **Reduce nightly THC:** Measure baseline
- **Start knee-smart training:** Pain-light baseline
- **Continue enclomiphene as prescribed:** Keep dose stable

1 weeks

- **Build protein-fiber meals:** Viscous fiber
- **Reduce nightly THC:** Step down

2 weeks

- **Build protein-fiber meals:** Refine carbs
- **Add cardio and lifting:** Add strength sessions
- **Track your home BP:** Review pattern
- **Track your home BP:** Review trend
- **Set a fixed wake time:** Protect consistency

4 weeks

- **Add cardio and lifting:** Interval add-on
- **Add cardio and lifting:** Progress aerobic volume
- **Add cardio and lifting:** Progress volume
- **Start knee-smart training:** Quad-hip build
- **Continue enclomiphene as prescribed:** Standardize lab timing

6 weeks

- **Track your home BP:** Recheck effect
- **Reduce nightly THC:** Reassess sleep

8 weeks

- **Build protein-fiber meals:** Lipid review
- **Start knee-smart training:** Fourth session
- **Continue enclomiphene as prescribed:** Review response

12 weeks

- **Add cardio and lifting:** Progression

Your Coach

Here's your personalized guide for each part of your plan — what to take, when, and how to get started.

Build protein-fiber meals

Protein-Fiber Plate

WHAT TO DO

Build meals from protein first, then vegetables or other high-fiber foods, and keep refined starches and sugars as the smallest part of the plate.

WHY IT MATTERS

Your last fasting pattern suggests your body was using excess insulin to hold glucose in range. Reducing that insulin demand is the fastest way to improve both metabolic risk and fatty-liver risk.

HOW IT WORKS

Protein and fiber slow absorption, reduce post-meal glucose swings, improve fullness, and reduce the liver's drive to package excess energy into triglycerides.

WEEK 1 PLAN

At two meals per day, make protein the anchor. Add one fiber-rich food at each meal. Remove one obvious source of sugar or refined starch that you eat most often.

WHAT TO EAT / WHAT TO AVOID

Prioritize eggs, fish, Greek yogurt, tofu, poultry, beans, lentils, vegetables, berries, nuts, seeds, and minimally processed starches. Cut back on sugary drinks, desserts, large evening starch loads, and ultra-processed snack foods.

COMMON QUESTIONS

Do I need to go very low carb? No. The goal is a lower-insulin pattern, not a rigid diet label.

What if my A1c is already 5.5%? Your insulin and triglycerides show there is still metabolic work happening behind that A1c.

TIP

If you are unsure where to start, change breakfast first; it often sets the insulin pattern for the rest of the day.

Add cardio and lifting

Zone 2 plus lifting

WHAT TO DO

Build a weekly routine that combines steady aerobic work with resistance training.

WHY IT MATTERS

This is one of the most reliable ways to lower insulin resistance before diabetes develops. It also helps triglycerides, body composition, and liver fat risk.

HOW IT WORKS

Working muscles take up more glucose and become more sensitive to insulin. Over time, your body does not need to over-secrete insulin to keep glucose normal.

WEEK 1 PLAN

Do three brisk walks or bike sessions of 30-45 minutes and two short full-body strength sessions.

COMMON QUESTIONS

Do I need intense exercise? No. Consistent moderate work plus basic strength training is enough to start changing the biology.

What if I am deconditioned? Start shorter and build; consistency matters more than intensity in week one.

TIP

Schedule exercise right after work or before dinner so it becomes part of the same daily slot.

Track your home BP

Track home BP

WHAT TO DO

Measure and log your blood pressure at home in a standardized way for the next few weeks.

WHY IT MATTERS

You now have a documented hypertension diagnosis, but there are still no actual readings in the chart. With stimulant exposure on board, guessing is not good enough.

HOW IT WORKS

Repeated home readings give a much better picture of your real blood-pressure load than an isolated office impression and help your clinician see whether treatment is actually holding.

WEEK 1 PLAN

Set up a validated cuff, measure twice each morning and evening on at least 4 days, and store the readings in one place you can share.

COMMON QUESTIONS

Q: What if one reading is high? A: Look at the pattern, not a single number, unless you have symptoms or very severe values that need urgent care.

Q: Should I measure right after exercise or caffeine? A: No. Measure when rested and consistent so the log is interpretable.

TIP

Keep the cuff next to something you already do every morning and evening so the routine sticks.

Set a fixed wake time

Fixed Wake Time

WHAT TO DO

Wake up at the same time every day for the next month.

WHY IT MATTERS

This is the fastest way to steady a disrupted sleep schedule when medications, stress, or evening habits are pushing your brain in different directions.

HOW IT WORKS

A fixed wake time strengthens your circadian rhythm and helps sleep pressure build at the right time the next night.

WEEK 1 PLAN

Pick one realistic wake time, set alarms, and get out of bed within 10 minutes every day.

COMMON QUESTIONS

What if I slept badly? Keep the wake time anyway; consistency matters more than one rough night.

TIP

Put the alarm across the room so getting up becomes automatic.

Reduce nightly THC

THC Taper

WHAT TO DO

Reduce nightly THC use in a gradual, trackable way instead of using it automatically every evening.

WHY IT MATTERS

Your updated chart now shows nightly THC on top of stimulant and antidepressant treatment, which is a setup for sleep that may feel sedated but not fully restorative.

HOW IT WORKS

Regular THC can blunt REM sleep and create rebound sleep disruption as tolerance develops. Lowering the exposure helps your sleep architecture recover.

WEEK 1 PLAN

Write down your usual nightly pattern, then choose one small step-down target for the next week.

COMMON QUESTIONS

Do I have to stop all at once? No. A gradual reduction is usually more sustainable and gives cleaner feedback.

TIP

Track next-morning clarity, dream recall, and energy so the benefits become easier to notice.

Start knee-smart training

Knee-smart strength plan

WHAT TO DO

Keep lifting, but make your lower-body training more knee-efficient: controlled tempo, moderate range, and gradual progression.

WHY IT MATTERS

Strong quads, glutes, and hamstrings help unload the knee and make osteoarthritis less limiting over time.

HOW IT WORKS

Better muscle support and cleaner movement patterns reduce joint irritation while preserving the training habit that protects the rest of your health.

WEEK 1 PLAN

Keep your current three gym sessions. On lower-body days, reduce any movement that causes a clear pain spike, favor split squats, box squats, leg press in a tolerated range, hamstring work, and step-ups.

WHAT TO EAT / WHAT TO AVOID

Prioritize protein-containing meals around training and avoid letting post-workout meals turn into low-protein convenience eating.

COMMON QUESTIONS

Should you stop squatting? No—usually the better move is to change depth, tempo, stance, or load.

Is pain ever acceptable? Mild, short-lived discomfort can be workable; pain that clearly worsens during the session or lingers into the next day means back down.

TIP

Change only one loading variable at a time—weight, depth, or volume, not all three together.

Continue enclomiphene as prescribed

Continue enclomiphene

WHAT TO DO

Keep taking enclomiphene exactly as currently prescribed and repeat your hormone surveillance labs in the next 1-3 months.

WHY IT MATTERS

Your recent records make it much more likely that the spring testosterone rise was the intended treatment effect. The main question now is whether you are getting that benefit without letting estradiol drift too high.

HOW IT WORKS

This medication helps your brain send a stronger signal to your testes to produce testosterone. That same shift can also raise estradiol, so follow-up labs are how you keep the benefits while avoiding overshooting.

WEEK 1 PLAN

Keep your morning dosing routine consistent, note any breast tenderness, fluid retention, acne, mood change, headaches, or libido changes, and book the follow-up lab draw.

COMMON QUESTIONS

If testosterone looks good, why recheck anything? Because estradiol, hematocrit, and liver chemistry help show whether the treatment is still balanced and safe.

Should I change the dose before the next lab? No—keeping the regimen stable makes the next result much easier to interpret.

TIP

Try to get future hormone labs drawn under the same conditions each time, especially with respect to your morning dose.

SAFETY

What to avoid:

- Do not continue this medication without clinician review if you develop unexplained vision changes, chest pain, shortness of breath, or one-sided leg swelling, or if you have been told you have an active clotting disorder or significant liver disease.

How we'll watch this:

- Repeat estradiol and the rest of your surveillance labs on schedule so your clinician can confirm the treatment stays balanced and safe. (we'll check Estradiol once in the next 1-3 months, then per clinician)

Dosing:

- Do not increase the dose on your own; stay at 25 mg each morning unless your prescribing clinician changes the regimen. Don't exceed 25 mg each morning unless prescribing clinician revises it; we'll titrate per Maintain current dose until the follow-up panel is reviewed.

Call us right away if:

- Stop the medication and contact your clinician promptly if you develop visual symptoms, severe persistent headache, marked mood worsening, breast tenderness with rapid swelling, or symptoms concerning for a blood clot.

Health Deep Dive



Cardiovascular Health

AT RISK

At risk due to newly documented essential hypertension requiring pharmacotherapy (propranolol), complicated by concurrent stimulant use (Adderall XR) and a strong family history of cardiovascular disease. The August 2025 clinic facesheet reveals this active disease state, which recontextualizes the patient's metabolic profile. Historical 2023-11-28 labs show a prior peak of severe atherogenic dyslipidemia (LDL-C 132 mg/dL, triglycerides 292 mg/dL) that has only partially improved on the 2025-06-09 fasting panel (LDL-C 101 mg/dL, triglycerides 187 mg/dL, insulin 27.8 uIU/mL). The immediate priority is obtaining objective home blood pressure logs to ensure the propranolol is adequately controlling hypertension against the stimulant's effects, alongside updating the stale lipid panel with ApoB and Lp(a) to finalize a prevention strategy.

● Abnormal (10) ● In Range (6) ● Optimal Range (2)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Triglycerides (latest 2025-06)	187 mg/dL	< 150	< 100	Jun 9, 2025
High	Insulin	27.8 µIU/mL	2–25	2–8	Jun 9, 2025
High	LDL-Cholesterol (latest 2025-06)	101 mg/dL	< 130	< 80	Jun 9, 2025
Medium	LDL-Cholesterol (baseline 2023-11)	132 mg/dL	< 130	< 80	Nov 28, 2023
Medium	HDL Cholesterol (latest 2025-06)	39 mg/dL	> 40	60–90	Jun 9, 2025
Medium	Triglycerides (baseline 2023-11)	292 mg/dL	< 150	< 100	Nov 28, 2023
Medium	Non-HDL Cholesterol (baseline 2023-11)	177 mg/dL	< 160	< 130	Nov 28, 2023
Medium	Hemoglobin A1c (baseline 2025-03)	5.7 %	4–5.6	4.8–5.2	Mar 27, 2025
Medium	Hemoglobin A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Medium	Glucose (latest 2025-06)	98 mg/dL	65–99	80–90	Jun 9, 2025

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
Medium	Non-HDL Cholesterol (latest 2025-06)	129 mg/dL	< 160	< 130	Jun 9, 2025
Medium	hs-CRP	0.5 mg/L	< 1	< 1	Jun 9, 2025
Low	Creatinine (baseline 2025-03)	1.48 mg/dL	0.6–1.3	0.8–1	Mar 27, 2025
Low	Creatinine (latest 2025-06)	1.33 mg/dL	0.6–1.3	0.8–1	Jun 9, 2025
Low	eGFR (latest 2025-06)	66 mL/min/1.73m ²	< 60	> 90	Jun 9, 2025
Low	HDL Cholesterol (baseline 2023-11)	40 mg/dL	> 40	60–90	Nov 28, 2023
Low	Glucose (baseline 2025-03)	92 mg/dL	65–99	80–90	Mar 27, 2025
Low	eGFR (baseline 2025-03)	58 mL/min/1.73 m ²	< 60	> 90	Mar 27, 2025



Metabolic Health

AT RISK

Metabolic profile is AT RISK, driven by a chronic hyperinsulinemic insulin-resistance pattern. The most recent fasting draw (2025-06-09) showed insulin 27.8 uIU/mL with glucose 98 mg/dL, HbA1c 5.5%, triglycerides 187 mg/dL, HDL-C 39 mg/dL, and LDL-C 101 mg/dL. Newly integrated historical data from late 2023 reveals this is a long-standing issue, previously manifesting as severe hypertriglyceridemia (292 mg/dL) and elevated ALT (85 U/L), strongly suggesting hepatic insulin resistance and metabolic-associated steatotic liver disease (MASLD). While HbA1c, lipids, and ALT improved by mid-2025, the dominant signal of compensatory hyperinsulinemia persists. Furthermore, the patient is prescribed propranolol, a beta-blocker known to potentially worsen glycemic control and lipid profiles. These data are now about 12 months old, warranting confirmation with repeat fasting metabolic labs.

● Abnormal (9) ● In Range (4) ● Optimal Range (3)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Insulin	27.8 µIU/mL	2–25	2–8	Jun 9, 2025
High	Triglycerides (baseline 2023-11)	292 mg/dL	< 150	< 100	Nov 28, 2023

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Triglycerides (latest 2025-06)	187 mg/dL	< 150	< 100	Jun 9, 2025
High	ALT (baseline 2023-11)	85 U/L	< 35	< 25	Nov 28, 2023
High	Hemoglobin A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
High	Glucose (latest 2025-06)	98 mg/dL	65–99	80–90	Jun 9, 2025
Medium	Hemoglobin A1c (baseline 2025-03)	5.7 %	4–5.6	4.8–5.2	Mar 27, 2025
Medium	HDL Cholesterol (latest 2025-06)	39 mg/dL	> 40	60–90	Jun 9, 2025
Medium	LDL-Cholesterol (baseline 2023-11)	132 mg/dL	< 130	< 80	Nov 28, 2023
Medium	Non-HDL Cholesterol (baseline 2023-11)	177 mg/dL	< 160	< 130	Nov 28, 2023
Medium	HDL Cholesterol (baseline 2023-11)	40 mg/dL	> 40	60–90	Nov 28, 2023
Medium	LDL-Cholesterol (latest 2025-06)	101 mg/dL	< 130	< 80	Jun 9, 2025
Medium	ALT (latest 2025-06)	20 U/L	< 35	< 25	Jun 9, 2025
Medium	Non-HDL Cholesterol (latest 2025-06)	129 mg/dL	< 160	< 130	Jun 9, 2025
Low	Glucose (baseline 2025-04)	102 mg/dL	65–99	80–90	Apr 10, 2025
Low	HS CRP	0.5 mg/L	< 1	< 1	Jun 9, 2025



Musculoskeletal Health

AT RISK

Musculoskeletal risk is driven by active joint pathology, specifically the newly documented bilateral primary osteoarthritis of the knees and a history of multiple arthroscopies. The patient's regular gym training (3 days/week) is a strong protective factor for muscle preservation and joint stabilization. Available 2025 laboratory data are modestly reassuring for mineral homeostasis and inflammatory burden: calcium remained normal at 9.7 mg/dL, alkaline phosphatase was low-normal at 43 U/L, and hsCRP was low at 0.5 mg/L. However, core longevity metrics—including 25-OH vitamin D, PTH, DXA for

bone density, and objective functional strength testing—remain absent, leaving bone and muscle reserve incompletely characterized.

● Abnormal (5) ● In Range (2) ● Optimal Range (2)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
Medium	eGFR (baseline 2023-11)	67 mL/min/1.73 m ²	< 60	> 90	Nov 28, 2023
Medium	eGFR (latest 2025-06)	66 mL/min/1.73m ²	< 60	> 90	Jun 9, 2025
Medium	Calcium (baseline 2023-11)	10.2 mg/dL	8.6–10.2	9.2–9.8	Nov 28, 2023
Medium	Calcium (latest 2025-06)	9.7 mg/dL	8.6–10.2	9.2–9.8	Jun 9, 2025
Medium	hsCRP	0.5 mg/L	< 1	< 1	Jun 9, 2025
Low	Alkaline Phosphatase (latest 2025-06)	43 U/L	44–147	50–90	Jun 9, 2025
Low	Creatinine (baseline 2023-11)	1.32 mg/dL	0.6–1.3	0.8–1	Nov 28, 2023
Low	Creatinine (latest 2025-06)	1.33 mg/dL	0.6–1.3	0.8–1	Jun 9, 2025
Low	Alkaline Phosphatase (baseline 2023-11)	123 U/L	44–147	50–90	Nov 28, 2023



Brain Health

AT RISK

Brain health is currently dominated by an active neuropsychiatric profile requiring polypharmacy (Adderall XR, citalopram, bupropion) for ADHD and depression, alongside migraine management (propranolol) and nightly THC use. This neuroactive medication burden is compounded by essential hypertension and a persistent vascular-metabolic risk pattern (fasting insulin 27.8 uIU/mL, triglycerides 187 mg/dL, LDL-C 101 mg/dL). While glycemic control has improved (HbA1c 5.5%) and systemic inflammation is low (hs-CRP 0.5 mg/L), the combination of psychiatric comorbidity, stimulant use, THC exposure, and vascular risk factors significantly elevates the risk for both short-term cognitive fluctuation and long-term vascular brain injury. Comprehensive medication reconciliation, strict blood pressure monitoring, and targeted lifestyle interventions are critical.

● Abnormal (5) ● In Range (4) ● Optimal Range (3)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Insulin	27.8 µIU/mL	2–25	2–8	Jun 9, 2025
High	Triglycerides	187 mg/dL	< 150	< 100	Jun 9, 2025
Medium	Hemoglobin A1c (baseline 2025-03)	5.7 %	4–5.6	4.8–5.2	Mar 27, 2025
Medium	HDL Cholesterol	39 mg/dL	> 40	60–90	Jun 9, 2025
Medium	Hemoglobin A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Medium	Glucose	98 mg/dL	65–99	80–90	Jun 9, 2025
Medium	LDL-Cholesterol	101 mg/dL	< 130	< 80	Jun 9, 2025
Low	eGFR (latest 2025-06)	66 mL/min/1.73m2	< 60	> 90	Jun 9, 2025
Low	eGFR (baseline 2025-03)	58 mL/min/1.73 m^2	< 60	> 90	Mar 27, 2025
Low	HS CRP	0.5 mg/L	< 1	< 1	Jun 9, 2025
Low	TSH (baseline 2025-03)	1.21 mIU/L	0.4–4.5	0.5–2.5	Mar 27, 2025
Low	TSH (latest 2025-06)	1.72 mIU/L	0.4–4.5	0.5–2.5	Jun 9, 2025



Emotional & Social Health

AT RISK

The patient's emotional and cognitive health profile is notable for active depression and attention-deficit hyperactivity disorder (ADHD), currently managed with a multi-drug regimen including citalopram, bupropion SR, and Adderall XR. The 2025-08-15 clinic facesheet also documents nightly THC use, which warrants careful monitoring given its potential to interact with psychiatric medications, impact sleep architecture, and exacerbate underlying mood or cognitive symptoms. As a physician who is divorced with two children, the patient likely faces significant occupational and psychosocial stressors. A family history of depression and anxiety on both sides further elevates risk. Comprehensive behavioral health follow-up is recommended to optimize the pharmacologic regimen, assess the efficacy of current therapies, and evaluate the impact of THC use.



Thyroid & Hormone Health

NEEDS ATTENTION

Thyroid signal is reassuring but incomplete: TSH was optimal at 1.21 mIU/L (2025-03-27) and 1.72 mIU/L (2025-06-09), with no available Free T4, Free T3, reverse T3, or thyroid antibodies to assess peripheral conversion or autoimmunity. The dominant endocrine feature is the male reproductive axis, where the previously noted volatility across spring 2025 is now clearly explained by the August 2025 clinic facesheet documenting a diagnosis of testicular hypofunction and active treatment with enclomiphene. The rise in total testosterone from 310 to 726 ng/dL, accompanied by elevated LH, FSH, and estradiol, represents the expected physiologic response to selective estrogen receptor modulator (SERM) therapy rather than an unstable endogenous axis. Ongoing surveillance is required to ensure clinical optimization and monitor for SERM-related effects.

● Abnormal (3) ● In Range (4) ● Optimal Range (6)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	LH (latest 2025-06)	14.9 mIU/mL	1.7–8.6	3–7	Jun 9, 2025
High	FSH (latest 2025-06)	16.6 mIU/mL	1.5–12.4	2–8	Jun 9, 2025
High	Testosterone, Total, MS (baseline 2025-03)	310 ng/dL	264–916	500–900	Mar 27, 2025
High	Testosterone, Total, MS (latest 2025-06)	726 ng/dL	264–916	500–900	Jun 9, 2025
High	Testosterone, Free	133.9 pg/mL	35–155	> 100	Jun 9, 2025
High	LH (baseline 2025-04)	3.9 mIU/L	1.7–8.6	3–7	Apr 10, 2025
High	FSH (baseline 2025-04)	7 mIU/L	1.5–12.4	2–8	Apr 10, 2025
Medium	Estradiol (latest 2025-06)	53 pg/mL	10–40	20–30	Jun 9, 2025
Medium	Sex Hormone Binding Globulin (baseline 2025-03)	20 nmol/L	10–57	30–40	Mar 27, 2025
Medium	Sex Hormone Binding Globulin (latest 2025-06)	24 nmol/L	10–57	30–40	Jun 9, 2025
Medium	Estradiol (baseline 2025-04)	18 pg/mL	10–40	20–30	Apr 10, 2025
Medium	TSH (baseline 2025-03)	1.21 mIU/L	0.4–4.5	0.5–2.5	Mar 27, 2025
Medium	TSH (latest 2025-06)	1.72 mIU/L	0.4–4.5	0.5–2.5	Jun 9, 2025

While the 2025 hepatic panel shows no active hepatocellular injury or synthetic dysfunction, newly integrated historical data from late 2023 reveals a prior ALT elevation of 85 U/L paired with severe hypertriglyceridemia. This confirms that the patient's metabolic profile has previously driven overt liver stress. The current dominant issue remains probable metabolic-associated steatotic liver disease (MASLD) risk, driven by fasting insulin of 27.8 uIU/mL, triglycerides of 187 mg/dL, and low HDL-C. Additionally, the patient is on a complex medication regimen (bupropion, citalopram, Adderall, propranolol, enclomiphene) alongside nightly THC use, all of which require hepatic metabolism. Because the most recent normal labs are now roughly a year old, a repeat hepatic panel (including GGT) and a first-line steatosis evaluation (ultrasound or FibroScan) are warranted.

● Abnormal (5) ● In Range (3) ● Optimal Range (7)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Insulin	27.8 μ U/mL	2–25	2–8	Jun 9, 2025
High	Triglycerides	187 mg/dL	< 150	< 100	Jun 9, 2025
High	ALT (baseline 2025-03)	34 U/L	< 35	< 25	Mar 27, 2025
High	AST (baseline 2025-03)	26 U/L	< 35	< 25	Mar 27, 2025
High	ALT (latest 2025-06)	20 U/L	< 35	< 25	Jun 9, 2025
High	AST (latest 2025-06)	16 U/L	< 35	< 25	Jun 9, 2025
Medium	Hemoglobin A1c (baseline 2025-03)	5.7 %	4–5.6	4.8–5.2	Mar 27, 2025
Medium	Alkaline phosphatase	43 U/L	44–147	50–90	Jun 9, 2025
Medium	Hemoglobin A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Medium	Bilirubin, total (latest 2025-06)	0.3 mg/dL	0.2–1.2	< 1	Jun 9, 2025
Medium	Albumin	4.3 g/dL	3.5–5.5	4.2–4.8	Jun 9, 2025
Medium	Platelet count	233 Thousand/uL	150–450	200–300	Jun 9, 2025
Medium	Bilirubin, total (baseline 2025-03)	0.3 mg/dL	0.2–1.2	< 1	Mar 27, 2025
Low	HDL cholesterol	39 mg/dL	> 40	60–90	Jun 9, 2025
Low	hs-CRP	0.5 mg/L	< 1	< 1	Jun 9, 2025



Kidney Health

NEEDS ATTENTION

Kidney function is not frankly failing, but it is suboptimal for longevity and compounded by newly documented risk factors. Creatinine remained mildly elevated across 2025 and creatinine-based eGFR was 58 mL/min/1.73m2 in March 2025, improving to 66 by June 2025 and plateauing there; historical 2023 laboratory data confirm this is a chronic pattern. For a 49-year-old man, an eGFR in the mid-60s is below ideal. The August 2025 clinic facesheet adds critical context: active essential hypertension, a history of nephrolithiasis (2002), and a maternal family history of kidney disease. The key limitation is that the current driver is a June 2025 lab set, now just beyond a 12-month chemistry cadence, and a 2023 attempt at a timed urine albumin test was invalid due to missing volume data. Repeat creatinine/eGFR plus cystatin C, a valid spot urine albumin-creatinine ratio, urinalysis, and blood pressure logs are needed to distinguish benign creatinine elevation from early CKD and to set a nephroprotection plan.

● Abnormal (3) ● In Range (5) ● Optimal Range (2)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Creatinine (baseline 2025-03)	1.48 mg/dL	0.6–1.3	0.8–1	Mar 27, 2025
High	Creatinine (latest 2025-06)	1.33 mg/dL	0.6–1.3	0.8–1	Jun 9, 2025
High	eGFR (latest 2025-06)	66 mL/min/1.73m2	< 60	> 90	Jun 9, 2025
High	eGFR (baseline 2025-03)	58 mL/min/1.73 m^2	< 60	> 90	Mar 27, 2025
Medium	Urea Nitrogen (BUN)	18 mg/dL	7–25	10–15	Jun 9, 2025
Medium	Potassium	4.8 mmol/L	3.5–5.1	4–4.5	Jun 9, 2025
Medium	Carbon Dioxide	29 mmol/L	22–30	24–28	Jun 9, 2025
Medium	Hemoglobin A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Low	BUN/Creatinine Ratio	14 ratio	10–20	10–15	Jun 9, 2025
Low	Sodium	140 mmol/L	136–145	140–144	Jun 9, 2025



Nutrition & Vitamins

NEEDS ATTENTION

Nutrition status cannot be fully characterized because core micronutrient labs are absent, but the longitudinal metabolic pattern points to chronic nutritional optimization needs. The newly integrated 2023-11-28 data shows severe historical hypertriglyceridemia (292 mg/dL) that has partially improved by 2025-06-09 (187 mg/dL), yet fasting insulin remains markedly elevated at 27.8 uIU/mL. This is consistent with an insulin-resistant, atherogenic nutrition physiology despite HbA1c improving to 5.5%. The 2025-08-15 clinic facesheet notes regular exercise and no alcohol use—both strong metabolic assets—but nightly THC use may influence dietary choices and metabolic health. Because the latest nutrition-relevant labs are from mid-2025 and core micronutrients (vitamin D, B12, magnesium, iron, folate, omega-3) remain untested, confirmation testing is warranted.

● Abnormal (7) ● In Range (4) ● Optimal Range (2)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Insulin	27.8 µIU/mL	2–25	2–8	Jun 9, 2025
High	Triglycerides (baseline 2023-11)	292 mg/dL	< 150	< 100	Nov 28, 2023
High	Triglycerides (latest 2025-06)	187 mg/dL	< 150	< 100	Jun 9, 2025
High	Hemoglobin A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Medium	Hemoglobin A1c (baseline 2025-03)	5.7 %	4–5.6	4.8–5.2	Mar 27, 2025
Medium	HDL Cholesterol (latest 2025-06)	39 mg/dL	> 40	60–90	Jun 9, 2025
Medium	Glucose	98 mg/dL	65–99	80–90	Jun 9, 2025
Medium	HDL Cholesterol (baseline 2023-11)	40 mg/dL	> 40	60–90	Nov 28, 2023
Medium	hs-CRP	0.5 mg/L	< 1	< 1	Jun 9, 2025
Low	Non-HDL Cholesterol (baseline 2023-11)	177 mg/dL	< 160	< 130	Nov 28, 2023
Low	LDL-Cholesterol (baseline 2023-11)	132 mg/dL	< 130	< 80	Nov 28, 2023
Low	LDL-Cholesterol (latest 2025-06)	101 mg/dL	< 130	< 80	Jun 9, 2025
Low	Non-HDL Cholesterol (latest 2025-06)	129 mg/dL	< 160	< 130	Jun 9, 2025



Inflammation

NEEDS ATTENTION

Measured systemic inflammation via hsCRP appears low (0.5 mg/L on 2025-06-09), but the August 2025 clinic facesheet documents a diagnosis of psoriasis, indicating an active systemic autoimmune and inflammatory process. Furthermore, the inflammatory context is complicated by metabolic drivers: fasting insulin 27.8 uIU/mL, triglycerides 187 mg/dL (improved from 292 mg/dL in 2023), and low HDL-C 39 mg/dL support a metabolic inflammation pattern. The presence of clinical psoriasis and metabolic dysregulation precludes an optimal rating despite the low hsCRP. Repeat hsCRP with broader inflammatory characterization (ESR, ferritin) is warranted to assess current inflammatory tone.

● Abnormal (4) ● In Range (4) ● Optimal Range (2)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	INSULIN	27.8 μ U/mL	2–25	2–8	Jun 9, 2025
High	hsCRP	0.5 mg/L	< 1	< 1	Jun 9, 2025
Medium	TRIGLYCERIDES (baseline 2023-11)	292 mg/dL	< 150	< 100	Nov 28, 2023
Medium	TRIGLYCERIDES (latest 2025-06)	187 mg/dL	< 150	< 100	Jun 9, 2025
Medium	HDL CHOLESTEROL (latest 2025-06)	39 mg/dL	> 40	60–90	Jun 9, 2025
Medium	HEMOGLOBIN A1c (baseline 2023-11)	5.3 %	4–5.6	4.8–5.2	Nov 28, 2023
Medium	HEMOGLOBIN A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Medium	HDL CHOLESTEROL (baseline 2023-11)	40 mg/dL	> 40	60–90	Nov 28, 2023
Medium	White Blood Cell Count (latest 2025-06)	5.5 Thousand/uL	4–11	5–7	Jun 9, 2025
Low	White Blood Cell Count (baseline 2023-11)	7.4 $\times 10^3/\mu$ L	4–11	5–7	Nov 28, 2023



Sleep Health

NEEDS ATTENTION

While direct sleep metrics remain absent, new data from the August 2025 clinic facesheet reveals a potent combination of sleep-disrupting pharmacological and lifestyle exposures. The patient is utilizing nightly THC and citalopram (both established REM sleep suppressors), alongside Adderall XR and bupropion SR (which increase hyperarousal and risk sleep onset/maintenance insomnia), and propranolol (which can suppress endogenous melatonin). When combined with the established 2025 metabolic pattern of hyperinsulinemia and dyslipidemia, this profile strongly suggests compromised sleep architecture and elevated cardiometabolic risk. Comprehensive sleep assessment, including architecture evaluation and OSA screening, is highly indicated.

● Abnormal (4) ● In Range (1) ● Optimal Range (2)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
Medium	Insulin	27.8 µIU/mL	2–25	2–8	Jun 9, 2025
Medium	TRIGLYCERIDES	187 mg/dL	< 150	< 100	Jun 9, 2025
Medium	HEMOGLOBIN A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Low	HEMOGLOBIN A1c (baseline 2025-03)	5.7 %	4–5.6	4.8–5.2	Mar 27, 2025
Low	HDL CHOLESTEROL	39 mg/dL	> 40	60–90	Jun 9, 2025
Low	TSH (baseline 2025-03)	1.21 mIU/L	0.4–4.5	0.5–2.5	Mar 27, 2025
Low	TSH (latest 2025-06)	1.72 mIU/L	0.4–4.5	0.5–2.5	Jun 9, 2025



Skin Health

NEEDS ATTENTION

The recent clinic facesheet documents a diagnosis of psoriasis, establishing an active inflammatory skin condition that requires targeted management and surveillance. While systemic inflammation appears low (hs-CRP 0.5 mg/L), persistent hyperinsulinemia (fasting insulin 27.8 uIU/mL) suggests underlying metabolic stress that can exacerbate inflammatory dermatoses and impair tissue repair. A comprehensive dermatology evaluation is warranted to stage the psoriasis, review UV-exposure history, and perform a full-body skin cancer screening.

● Abnormal (2) ● In Range (1) ● Optimal Range (1)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
Medium	Insulin	27.8 µIU/mL	2–25	2–8	Jun 9, 2025

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
Medium	Hemoglobin A1c (latest 2025-06)	5.5 %	4–5.6	4.8–5.2	Jun 9, 2025
Low	Hemoglobin A1c (baseline 2025-03)	5.7 %	4–5.6	4.8–5.2	Mar 27, 2025
Low	HS CRP	0.5 mg/L	< 1	< 1	Jun 9, 2025



Blood Health

OPTIMAL

Blood counts are reassuring: CBC markers from 2025-06-09, now slightly beyond the usual 12-month cadence, show no anemia, leukocyte abnormality, or platelet disorder, and the historical trend dating back to 2023 is stable. Hemoglobin 15.0 g/dL, hematocrit 45.2%, MCV 96.2 fL, RDW 12.7%, WBC 5.5 K/uL, and platelets 233 K/uL support currently preserved hematologic status. Still, iron stores and hematinic status were not assessed; ferritin, iron panel, B12, folate, and ideally a CBC differential should be repeated/added on the next draw to confirm that the normal CBC is durable and nutritionally supported.

● In Range (1) ● Optimal Range (1)

* Relevancy reflects the impact of each biomarker on the clinical interpretation

RELEVANCY	BIOMARKER	VALUE	REFERENCE RANGE	OPTIMAL RANGE	DATE
High	Hemoglobin (baseline 2025-03)	15.6 g/dL	13.5–17.5	14–15	Mar 27, 2025
High	Hemoglobin (latest 2025-06)	15.0 g/dL	13.5–17.5	14–15	Jun 9, 2025



Cancer Screening

OPTIMAL

Cancer screening status is now partially documented and up to date for the most critical age-appropriate metric. The patient completed a normal screening colonoscopy in 2020, satisfying colorectal screening requirements until 2030. However, new family history data reveals pancreatic cancer (paternal grandfather), breast cancer (mother), and leukemia (paternal grandmother). This pedigree warrants a formal hereditary cancer risk assessment to determine if genetic testing or earlier/enhanced surveillance is indicated. As the patient approaches age 50, a shared decision-making discussion regarding baseline PSA testing for prostate cancer screening is also due. The patient is a non-smoker and is not eligible for low-dose CT lung cancer screening.